

DATED _____ 2026

MEDBURY HEALTHCARE GROUP
and
DR CHINWE KPADUWA
and
CONSULT FOR AFRICA

MEMORANDUM OF UNDERSTANDING

relating to the establishment of a joint aesthetics and plastic surgery company in Lagos

Draft for discussion. Private and confidential.

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PRINCIPAL TERMS

A summary for the Parties. The clauses govern; where this page and a clause differ, the clause prevails.

Party	What it commits	What it receives, and how it is protected
Medbury Healthcare Group	Funding to an aggregate ceiling of NGN 150m, all in (9.3). as working capital and equipment, plus NGN 97m of rental income forgone (9.3, 9.6). The Lyfe Place base and its fit-out (6.2). The clinical and aesthetic equipment (6.2). The Nigerian corporate platform and back office (6.2). Institutional access, and referral of all Group aesthetics work to the Company (13.1).	60% of the Company (7.1). Its funding repaid in full, with a priority return, before any profit is shared with anyone (9.5). Rent for the base and terms for the equipment, so the investment is recovered and not written off (9.6). Approval of every budget before a naira is drawn, and of any spend above the threshold (9.4). Two of four board seats and, because tier two needs three votes, a position on which it cannot be outvoted on any operating decision (11.2). A veto on the name, dilution, scope, sale and any new site (10.4).
Dr Chinwe Kpaduwa	The clinical standard, protocols, technique and patient selection criteria (6.3). A royalty-free Nigerian licence of her name and marks, procured from KP Plastics (5.2). Training and credentialing of every clinician the Company employs (6.3). Personal delivery of the advanced and surgical work, once registered (6.3, 15.7). Her following, and the enquiry and audience data of her practice (6.3).	30% of the Company, issued for non-cash consideration, with no cash subscription asked of her (7.1, 7.4). Payment by the Company for clinical work, separate from and additional to any shareholder return (9.2). Sole authority over the clinical standard, credentialing, patient selection, and every use of her name (10.2). A seat on the Board as Clinical Director. Ownership of her marks retained throughout, with reversion if the venture ends (5.2). The casting view on the Company's name (4.4).
Consult for Africa	Structuring, incorporation and the agreements (6.4). Regulatory establishment, without which the Company cannot lawfully treat a patient (6.4, 15.10). Recruitment, operating systems, marketing operations, procurement and programme management (6.4, Schedule 1).	10% of the Company (7.1). A services agreement at arm's length, approved by the other two Parties with CFA not voting (9.7). One board seat, and expressly no deciding vote in any disagreement between the other two (11.4).

The matters still open are listed at Schedule 3, and none of them prevents the work at Schedule 2 from starting.

THIS MEMORANDUM OF UNDERSTANDING is made on _____ 2026

BETWEEN:

- (1) **MEDBURY HEALTHCARE GROUP**, of [registered address], represented by Dr Itunu Akinware, Group Chief Executive (“**Medbury**”);
- (2) **DR CHINWE KPADUWA**, of [address], a citizen of the Federal Republic of Nigeria (“**Dr Kpaduwa**”); and
- (3) **CONSULT FOR AFRICA**, of [registered address], represented by Dr Adebawale Odulana, Founding Partner (“**CFA**”),

each a “**Party**” and together the “**Parties**”.

BACKGROUND

- (A) The Parties wish to establish a company in Nigeria to carry on a premium aesthetics and plastic surgery practice in Lagos.
- (B) On 25 and 26 August 2026 the Parties agreed the structure of that venture: a single company held by all three Parties, separate from the Los Angeles practice of Dr Kpaduwa, having its own board, management, team and assets, carrying its own name, and clinically led by Dr Kpaduwa.
- (C) Dr Kpaduwa holds the Marks through KP Plastics, her United States entity. KP Plastics is not a shareholder in the Company; it licenses the Marks to the Company under clause 5.2, and Dr Kpaduwa shall procure its execution of that licence.
- (D) This Memorandum records the terms so agreed, sets out at Schedule 2 the work each Party is to undertake pending incorporation, and identifies at Schedule 3 the matters to be settled in the Shareholders' Agreement.
- (E) Save for the provisions identified in clause 17.1, this Memorandum is not intended to be legally binding.

IT IS AGREED as follows:

1. DEFINITIONS AND INTERPRETATION

- 1.1 In this Memorandum, unless the context otherwise requires:

Board	the board of directors of the Company, constituted under clause 10
Business	the business described in clause 3.2
Clinical Director	Dr Chinwe Kpaduwa, in her capacity as clinical director of the Company
Company	the company to be incorporated under clause 3.1
Group	Medbury and its subsidiaries and affiliated businesses from time to time
KP Plastic Surgery	the United States practice and entity of that name controlled by Dr Kpaduwa, the United States practice and entity controlled by Dr Kpaduwa, which owns the Marks and is the licensor under clause 5.2. KP Plastics is not a shareholder in the Company
House Brand	the name, marks and identity of the Company, selected under clause 4
Marks	the “KP Plastic Surgery”, “KP Plastics” and “Dr Chinwe Kpaduwa” names and the trade marks associated with them, owned by KP Plastic Surgery
Review Date	the date in November 2026 on which the Parties conduct the review under clause 14.2
Shareholders' Agreement	the shareholders' agreement and the constitution of the Company to be entered into by the Parties
Stage 1, Stage 2 and Stage 3	the stages described in clause 14.1

1.2 Clause and Schedule headings do not affect interpretation. A reference to a clause or a Schedule is to a clause of or a Schedule to this Memorandum, and the Schedules form part of it. Words in the singular include the plural and the converse.

1.3 Each matter is dealt with in one place only. Where a clause refers to another, the provision referred to governs and is not repeated.

2. **PURPOSE**

2.1 The purpose of the Company is to build a premium, woman-led aesthetics and plastic surgery institution in Lagos, offering a full-body service with a developed focus on the face, delivered to the standard in clause 15.1, and to build the team, the protocols and the systems by which that standard is delivered consistently.

2.2 The Company shall be developed on two principles, held together: that the product is proved before it is scaled, so that the standard is not traded for growth; and that what is built is capable of being scaled once proved, so that the investment stands on its own and the standard reaches more patients than the practice of any single clinician could.

3. **THE COMPANY**

3.1 The Parties shall procure the incorporation in Nigeria of a private company limited by shares under the Companies and Allied Matters Act 2020, having its registered office in Lagos.

3.2 The Business of the Company shall be premium aesthetics and plastic surgery, comprising injectable treatments, regenerative treatments, skin treatments, medical dermatology, aesthetic assessment and treatment planning, aftercare and the management of complications, cosmetic and plastic surgery delivered at licensed partner theatres under use-of-facility arrangements and

- operating privileges, and the training and credentialing of the Company's own clinicians.
- 3.3 The Business does not extend to the Los Angeles practice, patients, entity or brand of Dr Kpaduwa; to the general wellness, occupational health, diagnostic or pharmaceutical activities of the Group; to the ownership or operation of an inpatient hospital; or to any treatment which the Company cannot deliver to the standard in clause 15.1 and safely manage the complications of.
- 3.4 The permanent base of the Company shall be at The Lyfe Place, opening in Stage 2. During Stage 1 the Company shall trade from an existing Medbury facility identified by the Board, so that trading may begin without waiting for the base, and non-surgical treatment shall be delivered there. Procedures requiring a theatre, anaesthesia or overnight recovery shall be performed at a licensed partner facility under clause 15.11 throughout, both before and after the base opens. Occupancy of any Medbury premises, whether the Stage 1 facility or the base, is on the terms in clause 9.14.
- 3.5 The Company shall be wholly separate from the Los Angeles practice of Dr Kpaduwa, having its own board, management, employees, assets, bank accounts, books and liabilities. Neither shall be liable for the obligations of the other, and any collaboration between them shall be by written agreement.
- 3.6 The Company shall not be a department or division of the Group. It shall have its own board, its own executive and its own accounts, and requests from the Group shall be made through the Board.
- 3.7 The Company shall employ its own personnel. Where Medbury seconds any person to the Company it shall do so on written terms approved by the Board.
4. **NAME**
- 4.1 The Company shall be incorporated and shall trade under the House Brand, being a name of its own and distinct from the Marks, so that neither the Los Angeles practice nor the Company is confused with or prejudiced by the other, and so that the Company owns a name that is genuinely its own.
- 4.2 The House Brand shall be capable of registration at the Corporate Affairs Commission and as a trade mark in Nigeria; clearly distinct from the Marks; consistent with a woman-led, natural-result and boutique clinical proposition; and capable of carrying more than one site and more than one clinician in due course.
- 4.3 CFA shall prepare and circulate to the Parties, within ten business days of the date of this Memorandum, a shortlist of candidate names together with availability searches at the Corporate Affairs Commission and the Trade Marks Registry.
- 4.4 The Parties shall agree the House Brand. Where the Parties do not converge it shall be determined by the Board as a tier two matter under clause 10.6, the House Brand being an enduring asset of the Company rather than a clinical question. The Clinical Director's approval of the use of her own Marks under clause 5.2(c), and of clinical claims under clause 10.2(e), is unaffected and is not a power over the House Brand.
- 4.5 On selection the Company shall register the House Brand as a trade mark in Nigeria in the classes covering medical, surgical and beauty services, and shall register the corresponding domain names and social media handles.

5. INTELLECTUAL PROPERTY AND DATA

- 5.1 The Company shall own absolutely and in perpetuity the House Brand and its identity; the protocols and standard operating procedures as adapted for and documented within the Company; the training curriculum and the credentialing records; the patient records and database; the booking and operating systems as configured for the Company; the marketing content and photography produced by or for the Company, subject to clause 15.2; and the Company's supplier terms.
- 5.2 The Marks, and the underlying clinical know-how, technique and standard brought by Dr Kpaduwa, shall remain the property of KP Plastic Surgery and shall be licensed to the Company under a written trade mark and know-how licence executed alongside the Shareholders' Agreement, on the following terms:
- (a) the licence shall be exclusive for Nigeria, royalty free for its term and limited to the Business;
 - (b) the Marks shall be used in endorsement and not in substitution, the Company being held out as founded and clinically led by Dr Chinwe Kpaduwa of KP Plastic Surgery and never as the Los Angeles practice;
 - (c) KP Plastic Surgery shall approve each use of the Marks and may withdraw approval for any use falling below its standard;
 - (d) the Company shall not use the Marks outside Nigeria, and KP Plastics shall not license the Marks to any competing service in Nigeria during the term; and
 - (e) the licence terminates automatically on the earliest of: her ceasing to be Clinical Director; her ceasing active participation for a continuous period agreed in the Shareholders' Agreement; material breach by the Company that is not remedied; failure by the Company to maintain the clinical standard at clause 15.1; a change of control to which she has not consented; the Company's insolvency; or non-use of the Marks for an agreed period. Her vested equity survives termination of the licence; and
 - (f) on termination the Marks shall be withdrawn from the Company's use within an agreed wind-down period and the Company shall continue under the House Brand.
- 5.3 The Parties acknowledge that registration of the Marks outside Nigeria confers no rights within Nigeria, and that under the Trade Marks Act Cap T13, Laws of the Federation of Nigeria 2004 no proceeding lies to restrain the infringement of an unregistered trade mark. Accordingly KP Plastics shall, as a condition of the licence having practical effect, apply to register the Marks in Nigeria in the relevant classes, and the Company shall be recorded at the Trade Marks Registry as a registered user of the Marks under sections 33 to 35 of that Act. CFA shall first conduct availability searches, and if the Marks or a confusingly similar mark are already held by a third party in Nigeria the Parties shall reconsider the form of endorsement in clause 5.2(b).
- 5.4 **The boundary between what she brings and what the Company owns.** Background intellectual property, being everything Dr Kpaduwa or KP Plastic Surgery held before the venture together with any generalised improvement, technique, know-how, algorithm, protocol or training concept derived from her clinical expertise and capable of application outside the Company, remains hers and is licensed under clause 5.2. Company intellectual property, being workflows, administrative procedures, local forms, patient-facing materials and operational adaptations specific to the Company, belongs to the Company. Writing a technique down while working for the Company does not transfer it, and clause 5.1 is to be read subject to this clause.

- 5.5 CFA's platforms and systems remain the property of CFA and are licensed to the Company for the term under the services agreement. Ownership does not pass to the Company on any exit, and on termination the licence ends while the Company retains its own data, records and configuration.
- 5.6 That licence earns CFA no equity whatever. The fee at clause 9.16 buys the management and the systems together, so recognising the licence in the shareholding as well would be a double count on the same asset, and it is not recognised at Schedule 4.
- 5.7 What CFA does take equity for, at Schedule 4, is the configuration and implementation of those systems within the Company, being the build the Company keeps: the configuration, the data migration and the training. That is delivered work and is distinct from the licence.
- 5.8 Patient records and data shall be owned and controlled by the Company, held in the Company's systems and processed in accordance with the Nigeria Data Protection Act 2023 and the directives of the Nigeria Data Protection Commission. No Party shall take a copy for its own use, and no patient record shall be transferred out of the Company save as required by law or as directed by the patient.

6. CONTRIBUTIONS

- 6.1 This clause records what each Party contributes to the Company. It is distinct from clause 12 and Schedule 1, which record the Party accountable for each function of the Company from time to time.
- 6.2 Medbury shall contribute the establishment capital and working capital referred to in clause 9.3; the equipment on which the Business operates; the use of its own premises, being the Stage 1 facility and thereafter The Lyfe Place base, re-planned to accommodate the Business at material cost together with its fit-out; and the relationship with the partner hospital, which Medbury holds and negotiates and without which the Company has no theatre and no surgical revenue.
- 6.3 Medbury shall in addition provide Group referral flow under clause 13.2, transitional shared back office services under clause 13.7 if the Board so requires, and institutional standing and introductions to referring hospitals and corporate accounts. Those are not valued at Schedule 4 and earn no equity, because the Company pays for them at agreed rates under the clauses named. That is the same principle applied to every Party: what is paid for in cash is compensated, and what is compensated does not also earn equity.
- 6.4 **The Clinical Director shall act as the public face of the Company.** The Company is built on her credibility as much as on her technique, and lending the name without appearing behind it would give the Company the licence and not the benefit. She shall accordingly participate in the launch and in the public representation of the Company: named appearances at its events, participation in its social channels and press, interviews and speaking where reasonably requested, and the release of consented case material under clause 15.2. The extent is agreed in the Shareholders' Agreement, is measured with the other commitments at clause 7.15 and vests with them. This obligation sits alongside the training and the protocols and is not subordinate to them.
- 6.5 Dr Kpaduwa shall contribute the clinical authority on which the premium positioning of the Company rests, being the leadership of a United States trained and certified plastic surgeon, which the Company could not recruit locally; the licence of the Marks under clause 5.2; the clinical standard, protocols, technique, treatment algorithms and patient selection criteria constituting the founding standard of the Company; the training and credentialing of each clinician the Company employs, being the means by which the Company acquires its clinical capability; the personal delivery of the advanced and surgical work; introductions to her existing following on the basis in

clause 6.6; and brand and messaging direction.

- 6.6 **No database passes to the Company.** The patient, enquiry and audience data of KP Plastic Surgery remains its own and is not transferred, disclosed or made available to the Company. KP Plastic Surgery may communicate with its own audience and invite interested individuals to opt in to communications from the Company, and only the data of those who so opt in comes to the Company, on its own lawful basis. Nothing in this Memorandum requires the disclosure of personal data in breach of any law binding on KP Plastic Surgery.
- 6.7 CFA shall contribute the structuring of the venture, including this Memorandum, the Shareholders' Agreement, the licence under clause 5.2 and the incorporation of the Company; the regulatory establishment of the Company under clause 15.11, without which the Company cannot lawfully treat any patient; a specialist healthcare recruitment and leadership assessment capability; the operating systems on which the Company runs; a supply chain for imported consumables together with the management of foreign exchange exposure and lead times; and the operating team by which the Company is established and run.

7. SHARE CAPITAL

- 7.1 The issued share capital of the Company shall on incorporation be held as follows, derived as set out at Schedule 4.

Shareholder	Holding	Issued in consideration of
Medbury Healthcare Group	60%	capital, the facility, equipment, the corporate platform and market access
Dr Chinwe Kpaduwa	30%	the licence of the Marks, clinical leadership, the founding standard and the training
Consult for Africa	10%	structuring and establishment of the Company and the continuing operator role
Total	100%	

- 7.2 The percentages at clause 7.1 are the output of the contribution framework agreed between the Parties, under which each contribution is valued at market over three years, reduced by whatever cash the contributing Party is paid for it, and equity follows the residual.
- 7.3 They are computed on two settings, each of which is a choice belonging to the Party affected. The Clinical Director draws 50 per cent of a market surgeon's fee in cash under clause 9.2, the balance being a contribution to the Company. CFA charges the full market operating fee of 10 per cent of revenue under clause 9.7 and therefore takes no equity at all for operating the Company. If either election changes, the percentages change with it: a Party that takes more cash contributes less and holds less, and the converse. The Parties shall settle both elections before the Shareholders' Agreement is executed and the percentages shall be recomputed on the elections actually made.
- 7.4 Medbury's holding is carried by two things and not one. Its cash is capped at NGN 150m under clause 9.3, and cash at that level alone would support about 56 per cent. The balance is bought with rental income forgone: Medbury waives 45 per cent of the market rent on the premises the Company occupies, being NGN 97m across the window rather than the NGN 43m a lighter waiver would give. That is the hybrid of paying something and forgoing the rest, and it relieves the Company's profit and loss in the years it can least afford rent.

- 7.5 No Party is asked to accept the percentages on trust. Schedule 4 shows every line, the basis on which it is valued, the cash netted against it and the arithmetic that produces the result.
- 7.6 Contributions of income forgone accrue over a defined window. The window opens on the date the Company begins trading, because income cannot be forgone before there is income, and closes on the earlier of Medbury's capital being returned in full and the 3rd anniversary of the start of trading. Value delivered before trading begins is not forgone income but an asset brought to the Company, and is dealt with as such.
- 7.7 The longstop matters. The percentages are a function of how long the window runs, and without an outside date they could not be fixed at all. The Parties record that EBITDA turning positive does not close the window. It arrives well before any capital is repaid, and closing on it would end the accrual early and understate the contribution of the Parties forgoing income. What EBITDA turning positive does, once it has been sustained for four consecutive quarters, is trigger a step-up of cash compensation toward market rates, the Company being able by then to afford it. Cash changes when the business can pay; equity stops accruing when the risk is retired.
- 7.8 Because the window may close before the longstop, the percentages at clause 7.1 are the position expected on the current forecast and are not a fixed cap table. Equity referable to income forgone shall be issued in tranches against the amounts actually forgone, computed at each anniversary on actual revenue rather than forecast revenue, and shall cease to accrue when the window closes. Equity referable to assets brought and to capital invested is not affected and is fixed at incorporation.
- 7.9 Dr Kpaduwa is a citizen of the Federal Republic of Nigeria. Her shares are accordingly held by a Nigerian shareholder, and the Company is not a company with foreign participation. The Parties record that this is deliberate and material: had the holding been taken by KP Plastics as a United States entity, the Company would have required issued share capital of not less than NGN 100,000,000 under the Revised Handbook on Expatriate Quota Administration 2022 as applied by the Corporate Affairs Commission, fully paid up before any Business Permit or expatriate quota, together with registration at the Nigerian Investment Promotion Commission, a Business Permit and a Certificate of Capital Importation. None of those requirements arises on the structure in clause 7.1.
- 7.10 No share in the Company shall be issued or transferred to a non-Nigerian person or entity, and KP Plastics shall not take shares in the Company, without the prior unanimous agreement of the shareholders under clause 10.7(b), the Parties having regard to the consequences in clause 7.9. If Dr Kpaduwa prefers to hold through a company, it shall be a Nigerian company owned by her, which preserves the position in this clause.
- 7.11 Shares may be paid up in cash or by a valuable consideration other than cash under section 160 of the Companies and Allied Matters Act 2020. The shares issued in respect of the matters recorded against KP Plastics at clause 7.1 shall be issued for non-cash consideration, and no cash subscription is required of KP Plastics. The requirement in section 162 of that Act for an independent valuer to determine the value of consideration other than cash applies to public companies only and does not apply to the Company.
- 7.12 If any Party subscribes cash in excess of the contribution recorded against it in clause 6, the percentages shall be adjusted by agreement to reflect it.
- 7.13 **Not all of the Clinical Director's holding is contingent.** Equity referable to value delivered at or before launch, being the licence of the Marks, the clinical standard, the protocols and the founding clinical architecture, vests in full on incorporation and is not subject to any milestone. Only equity referable to services yet to be performed vests over time, and the measures against which it vests

shall be objective and written, and they are at Schedule 4A. No milestone may be introduced afterwards without her consent.

7.14 A holding at clause 7.1 is the figure reached on full service across the window. It is a ceiling rather than a guarantee. Each Party vests against what it has actually contributed in each year, and a Party that ceases to contribute keeps what it has vested and no more, the other holdings adjusting to match.

7.15 **Each holding comprises two parts.** The first is issued at incorporation, in consideration of capital already advanced and of assets already delivered to the Company. The second vests as the contribution it represents is actually made, and does not vest before. No Party receives equity in advance for a thing it has not yet done. The proportions follow from Schedule 4 and are:

Shareholder	Holding	Issued at once	Vesting	Of which vests
Medbury Healthcare Group	60%	NGN 126m	NGN 109m	46%
Dr Chinwe Kpaduwa	30%	NGN 45m	NGN 108m	71%
Consult for Africa	10%	NGN 18m	NGN 26m	60%

7.16 Equity vests as follows, and the Board shall record each vesting in the register:

- (a) **Medbury.** Equity referable to capital vests as that capital is actually drawn against the approved budget, and equity referable to rent forgone vests as the rent is actually forgone. Medbury is not credited on day one for money it has not yet advanced.
- (b) **Dr Kpaduwa.** Equity referable to the clinical standard and protocols is issued at incorporation upon their delivery to and documentation within the Company. Equity referable to the licence of the Marks, to training and credentialing, and to clinical fees forgone vests annually against, respectively, the licence remaining in force and registered under clause 5.3, the training and credentialing milestones being met, and the fees actually forgone in that year.
- (c) **CFA.** Equity vests on the Board's written acceptance of each founding deliverable, being incorporation and the statutory registrations; the regulatory establishment under clause 15.10; the founding team recruited and in post, including the resident surgeon before any surgical procedure is offered; and the operating systems live and configured. Nothing vests on incorporation alone.

7.17 The milestones against which Dr Kpaduwa's vesting is measured shall be settled in the Shareholders' Agreement and shall include her registration with the Medical and Dental Council of Nigeria on whichever route clause 15.5 establishes; the delivery of the training and credentialing programme to each clinician against the documented standard; the minimum clinical presence agreed under that Agreement; and the transfer and documentation of the protocols within the Company.

7.18 **Each Party has one core obligation, and the consequences of not meeting it are the same for all three.** Medbury's is to fund to the ceiling in clause 9.3 against the approved budget. The Clinical Director's is to provide the clinical leadership, training and presence agreed in the Shareholders' Agreement. CFA's is to establish and operate the Company to the milestones and service levels in its services agreement. A Party that does not meet its core obligation stops accruing the equity referable to it, and the Board shall record that at the next true-up under clause 8.7.

- 7.19 **Amicable exit.** Any Party may give six months' written notice that it wishes to withdraw, without fault and without needing a reason. On expiry it ceases to accrue equity, keeps what it has vested, and its vested shares are offered first to the remaining Parties in proportion to their holdings at a valuation agreed between them or determined by an independent valuer, failing which they may be retained as a passive holding carrying no board seat and no tier three vote. A Party leaving this way is not a bad leaver and forfeits nothing it has earned.
- 7.20 A withdrawal notice given before the review under clause 14.2 does not oblige the remaining Parties to continue, and they may instead discontinue under clause 14.3, in which case clause 9.4 governs what happens to the money.
- 7.21 **If a Party ceases to participate.** Equity that has vested is retained. Equity that has not vested lapses and does not vest afterwards, and the holdings of the remaining Parties adjust accordingly. Where a Party is in serious and defined breach, its vested equity may in addition be acquired at the lower of cost and value. The definition of serious breach shall be narrow and is settled in the Shareholders' Agreement, together with the treatment of a departure that is agreed rather than in breach.
- 7.22 For the avoidance of doubt, vesting applies to every Party on the same principle. It is not a condition imposed on one of them.

8. **BALANCE OF THE FRAMEWORK**

- 8.1 The percentages at clause 7.1 are produced by a small number of inputs, and each of those inputs moves the answer. The inputs do not move it evenly, and most of them lie within the practical control of one Party. Medbury sets the budget and so influences when EBITDA turns; it owns the premises and so states the rent; it funds and so paces the drawdown. This clause exists so that no Party can move a lever that changes another Party's holding.
- 8.2 **The rental figure is evidenced before signature, not after.** Medbury's holding turns more on the rent it forgoes than on any other single input. Before this Memorandum is signed Medbury shall produce the measured area of the premises the Company will occupy and evidence of the market rate for comparable space, and the figure at Schedule 4 shall be restated to match. If the evidenced figure differs materially from the figure modelled, the percentages are recomputed on it before signature rather than tried up afterwards.
- 8.3 **The inputs are agreed between the Parties.** The figures at Schedule 4, being the rental value and area of the clinic, the revenue attributable to the name, the market surgeon rate, the management fee benchmark and the forecast revenue, are agreed between the Parties at signature and recorded there. No Party may substitute its own figure afterwards. Any change to them requires the unanimous agreement of the shareholders under clause 10.4.
- 8.4 **The step-up affects cash only.** The contribution referable to income forgone is fixed at the outset for the window and is earned by serving that window. It is not recomputed against the cash actually drawn after the step-up under clause 7.6. Were it otherwise, a Party able to influence the timing of the step-up could reduce another Party's holding by exercising a judgement about spending, which is not a basis on which anyone should hold equity.
- 8.5 **EBITDA is defined and certified.** It shall be computed on a basis agreed in the Shareholders' Agreement, excluding the effect of discretionary variations in expenditure, and certified by the Board before any step-up takes effect.
- 8.6 **Each holding is collared.** Because the percentages float on actual outcomes rather than forecast ones, each Party's holding is bounded at plus or minus 4 percentage points of its opening position at

clause 7.1, save that CFA's holding shall not in any event exceed 10 per cent, CFA having accepted that ceiling. Without a collar the inputs could carry a holding a long way from where it starts, and no Party could reasonably sign. Within the collar the arithmetic governs; at its edge the collar does.

Shareholder	Opening	Floor	Ceiling
Medbury Healthcare Group	60%	56%	64%
Dr Chinwe Kpaduwa	30%	26%	34%
Consult for Africa	10%	6%	10%

- 8.7 **The true-up is visible to everyone.** At each anniversary the Company shall recompute the schedule on actual figures, circulate it to all Parties in the same form at the same time, and allow twenty-one days for any Party to challenge it before it is acted on.

9. FUNDING AND FINANCIAL ARRANGEMENTS

- 9.1 All income from patients of the Company belongs to the Company and shall be banked to the Company's accounts. No Party shall invoice a patient of the Company directly.
- 9.2 Clinicians, including the Clinical Director, shall be remunerated by the Company for clinical work on terms agreed in writing before Stage 1 delivery begins. Such remuneration is separate from and additional to any return as a shareholder, and equity is not in substitution for payment for clinical work.
- 9.3 **Medbury's aggregate funding obligation, including cash, equipment and any other capital expenditure funded by Medbury, shall not exceed NGN 150,000,000.** The ceiling is all in and is not a cash-only figure, so that no separate equipment or facility obligation can be argued to sit above it. Schedule 4 models the whole of it, being NGN 72m of working capital and NGN 54m of equipment. That is the whole of Medbury's funding obligation and it is not open-ended. Drawdown is against the approved budget under clause 9.4, and the ceiling is a limit rather than an entitlement: the difference between it and the requirement at Schedule 6 is contingency and may not be drawn without a further Board approval identifying what it is for.
- 9.4 **Stage 1 is separately capped and separately protected.** Expenditure before the review under clause 14.2 shall not exceed NGN 45,000,000 without the unanimous agreement of the shareholders. Stage 1 is the exposed phase: Medbury advances cash while the other Parties are contributing in kind and the Clinical Director cannot yet lawfully treat, so the money at risk is Medbury's alone and the ceiling should reflect what proof actually costs rather than what the full build will.
- 9.5 Cash advanced by Medbury before the review ranks as a first-priority shareholder loan. If the Parties discontinue at the review, the Company's assets shall be realised or distributed and the proceeds applied first in repaying that loan with the return under clause 9.13, before anything is applied to any other Party. Equipment and fit-out acquired in Stage 1 revert to Medbury at written-down value at its election. Unvested equity lapses under clause 7.15, so no Party carries away equity for a venture that did not proceed.
- 9.6 The Parties have considered whether Stage 1 should instead be funded by all three in proportion to their holdings. That remains open and is listed at Schedule 3. It would materially change the position recorded at clause 7.11, under which no cash subscription is asked of the Clinical Director, and it should not be decided without her.

- 9.7 Costs incurred by a Party's own advisers, agencies or contractors are that Party's own unless the Board has approved them in advance as a Company cost within an approved budget.
- 9.8 Funding required beyond that ceiling is not an obligation of any Party. It shall first be offered to all shareholders in proportion to their holdings, on the same terms and at the same time.
- 9.9 A Party funding beyond its proportionate share may elect that the excess be treated either as shareholder debt, repayable ahead of distributions on the terms in clause 9.13, or as a subscription for new shares at a valuation agreed between the Parties or determined independently. A Party that does not take up its proportion is diluted accordingly, and no Party is obliged to follow its money.
- 9.10 **Fresh capital may not be used to dilute a Party rather than to fund the Company.** Any issue under the preceding clause requires a bona fide funding need identified in a Board-approved business case; a subscription price set by an independent valuation where the Parties do not agree it; identical terms offered to every shareholder; and not less than thirty days' notice. **The Clinical Director's holding shall not be reduced below twenty per cent by any issue in which she was not offered a genuine opportunity to participate on those terms,** and Medbury and CFA shall not together exceed seventy-four per cent by such an issue, so that the threshold at clause 10.8 cannot be reached without her.
- 9.11 Shares issued for fresh capital under the preceding clause fall outside the collar at clause 8.5. The collar bounds movement in the true-up of the original contribution assumptions; it is not intended to cap a Party that puts materially more cash at risk than was forecast, and a Party doing so is entitled to have that reflected.
- 9.12 No drawdown shall be made otherwise than against an establishment budget and a twelve month operating budget approved by the Board, and no expenditure outside an approved budget above NGN 5,000,000 shall be incurred without Board approval.
- 9.13 Distributable cash shall be applied in the following order: operating costs; repayment of Medbury's funding together with a priority return of 20 per cent per annum, simple and not compounded, accruing on each drawdown from the date it is made and ceasing on repayment; reserves and reinvestment approved by the Board; and thereafter distributions to shareholders in proportion to their holdings.
- 9.14 Medbury's investment in the base and the equipment is capital at risk and is recognised in the shareholding under clause 7.1. So that it is recovered rather than written off and the Company's accounts remain true, the Company shall occupy the base under a written lease or occupancy licence at an agreed rate and shall hold equipment provided by Medbury on agreed terms. Medbury shall carry out the fit-out works to the base as owner of the premises, to a specification agreed with the Board.
- 9.15 CFA shall be engaged by the Company under a separate written services agreement, approved by Medbury and Dr Kpaduwa as a related-party matter with CFA not voting.
- 9.16 The fee shall be 10 per cent of revenue. Bundled fees for a full operating mandate of this kind run at about 8 to 20 per cent of collections, so the rate is at the lower-middle of the market. It shall be evidenced by comparables before the Shareholders' Agreement is executed.
- 9.17 **That fee is split by timing rather than reduced.** A base of 4 per cent of revenue is payable monthly in cash from the first month. The balance of 6 per cent accrues as a subordinated payable and is not paid until Medbury's funding and priority return have been repaid in full under clause 9.5. The Company therefore pays 4 per cent, not 10, in the years before it is profitable, which is when the charge is hardest to carry.

- 9.18 The deferred balance is deferred and not waived, and the distinction is deliberate. Income forgone is a contribution under clause 7.2 and would increase CFA's holding. Income deferred is not, so CFA's holding is unaffected and the Company keeps the cash. If CFA is removed for defined breach the deferred balance is extinguished.
- 9.19 The base steps down as the Company's own staff assume the functions in Part C of Schedule 1, on a schedule set out in the services agreement. That agreement shall have a defined initial term, service levels and key performance indicators reviewed annually by the Board, rights of termination for failure against them, a cap on reimbursable costs, and an express list of what the fee covers.
- 9.20 CFA charges the base in cash and defers the balance. The deferred 6 per cent, being NGN 72m across the window, ranks behind Medbury's capital and priority return and is paid only if the venture succeeds. It is therefore not merely deferred income but subordinated risk capital, and CFA carries the same risk of never being paid that a funder carries. That risk is recognised at Schedule 4 and nowhere else: CFA takes no equity for the management itself.
- 9.21 Each Party shall submit, within fourteen days of the date of this Memorandum, a schedule of the costs incurred by it from 1 July 2026 in reliance on the venture. On incorporation the Board shall review each schedule and shall either reimburse the cost from the Company or credit it to that Party's contribution, save that expenditure constituting investment in a Party's own asset shall be dealt with under clause 9.14. Undisputed items shall be settled within thirty days of incorporation, and no Party shall vote on its own claim.
- 9.22 No Party shall charge the Company for anything not provided for in an approved budget and a written agreement.

10. **DECISION MAKING**

- 10.1 Decisions of the Company fall into four tiers which do not overlap, the fourth being residual so that every decision has an owner. Every voting threshold in this Memorandum is fixed by this clause.
- 10.2 **Tier one.** The following are reserved to the Clinical Director alone and shall not be put to a vote:
- (a) the clinical standard and each treatment protocol;
 - (b) which treatments within the Business the Company offers and which it does not;
 - (c) the credentialing of each clinician, and the sign-off of each clinician before independent practice;
 - (d) patient selection, and the refusal of any patient or procedure on clinical grounds;
 - (e) each clinical claim made in the Company's marketing, and each use of the Marks; and
 - (f) the immediate suspension of any service on safety grounds, effective on notice and without prior Board approval.
- 10.3 The tier one matters are clinical. They do not extend to the commercial conduct of the business, and the following are tier two whatever their marketing consequence: the price list and pricing architecture; the marketing budget and the choice of channels; commercial positioning and the naming and packaging of services; opening hours; staffing numbers; and expansion. Approval of a clinical claim is not approval of a campaign, and the Clinical Director's authority over her own Marks under clause 5.2(c) is a right over the use of her name and not a veto on marketing.
- 10.4 Where the Clinical Director declines on clinical or ethical grounds to offer a treatment the Board wishes to offer, her decision stands under clause 10.2(b) and clause 15.14, and the Company's plan and forecast are adjusted to the menu actually offered.

- 10.5 **No exercise by the Clinical Director, in good faith, of her clinical, patient safety, credentialing, ethical or professional judgment shall constitute a failure of contribution, reduce her vested or unvested equity, alter the valuation of her contribution at Schedule 4, or constitute a breach of any obligation under this Memorandum.** The Parties record that an earlier draft provided otherwise and that it was wrong to do so: a Clinical Director who is financially penalised for declining a procedure is not a safeguard but a hazard, and the premium the Company charges rests on her being free to say no.
- 10.6 **Tier two.** The following are reserved to the Board and carry on the affirmative votes of three of the four directors:
- (a) the annual budget, the business plan and the price list;
 - (b) capital expenditure, and expenditure above the threshold set under clause {9.4};
 - (c) the appointment and removal of the executive;
 - (d) host institution agreements, any lease or occupancy of premises, and any other material contract;
 - (e) marketing strategy and marketing expenditure, subject always to clause 10.2(e); and
 - (f) any related-party contract, on which the interested Party shall not vote.
- 10.7 **Tier three.** The following require the unanimous agreement of all shareholders, being matters that change what the Company fundamentally is:
- (a) any change to the House Brand or to the name of the Company;
 - (b) the admission of a new shareholder, and any issue of shares other than an issue for fresh capital under clause 9.5, which requires no unanimity so that funding cannot be blocked by a Party declining to provide it;
 - (c) any change to the Business as described in clause 3.2;
 - (d) any amendment to the constitution or to the Shareholders' Agreement; and
 - (e) the termination or material amendment of the licence granted under clause 5.2.
- 10.8 **Tier three A.** The following require the agreement of shareholders holding seventy-five per cent or more of the shares, and not unanimity:
- (a) the sale of the Company or of its business, or its winding up. Shareholders holding seventy-five per cent or more may require the remaining shareholders to participate in a bona fide arm's length sale on the same terms, and no shareholder may block such a sale. CFA gives up its own veto here, an operator holding a minority stake being the wrong party to prevent an exit the owners of the capital and the clinical practice both support;
 - (b) a sale to which the Clinical Director has not consented shall, on completion, terminate the licence of the Marks under clause 5.2 and every right of the Company to use her name, the Company continuing under the House Brand;
 - (c) the opening of any additional site, subject always to the clinical standard and to the Clinical Director's rights under clause 10.2, which are unaffected; and
 - (d) borrowing above a threshold agreed by the Parties, or the granting of security over the Company's assets.
- 10.9 **Deadlock.** Where a tier three or tier three A matter is not resolved within thirty days, it shall be referred to the principals, then to a jointly appointed mediator within a further thirty days. If it

remains unresolved and the matter is fundamental to the Company continuing, any shareholder may serve a notice offering to buy the others' shares at a stated price per share, and each recipient may either accept or itself buy at that price.

- 10.10 The Parties record that a mechanism of that kind favours the Party with the deepest pocket, and that they are not equally capitalised. It is accordingly available only after the escalation and mediation above have been exhausted; only for a matter that genuinely prevents the Company continuing; not at all during the first two years; and subject to a minimum price supported by an independent valuation, a right for a recipient to require third-party financing time of not less than ninety days, and the termination of the licence of the Marks under clause 5.2 if the Clinical Director is bought out under it. The full mechanics are settled in the Shareholders' Agreement.
- 10.11 **Protective consents.** The Parties record that Medbury appointing two directors and CFA one means the two of them together carry a tier two decision. So that the Clinical Director is not exposed on matters that bear directly on her, the following additionally require her written consent, not to be unreasonably withheld or delayed:
- (a) any change to her remuneration, or to the basis on which she is remunerated;
 - (b) any change to the clinical presence or time commitment expected of her;
 - (c) any change to her vesting, its measures, or the dates against which it is measured;
 - (d) any use of her name or the Marks beyond the endorsement form at clause 5.2(b);
 - (e) any material change to the clinical scope of the Business, or the withdrawal of a treatment she has approved;
 - (f) any reduction in clinical staffing or clinical resources below the level her protocols require for safe practice; and
 - (g) any change that materially increases her personal professional exposure.
- 10.12 Where she withholds consent she shall give her reason in writing, and the matter is referred to the principals under clause 10.9. A refusal on grounds of patient safety or professional obligation is conclusive and is protected by clause 10.5.
- 10.13 **Tier four.** Every decision not falling within clauses 10.2 to 10.6 is a management decision, to be taken by the executive within the approved budget and business plan and reported to the Board.

11. THE BOARD

- 11.1 The Board shall comprise four directors. Medbury shall appoint two, Dr Kpaduwa one and CFA one. Dr Kpaduwa shall take her seat as Clinical Director.
- 11.2 The chair shall be appointed by Medbury and shall not have a casting vote. The rule operates in both directions, and the Parties record its effect so that it is not mistaken. No Party may carry a tier two decision alone. Equally, because a tier two decision requires three of four votes and Medbury appoints two directors, no tier two decision can be taken against Medbury's wishes. Medbury therefore holds, on every operating decision of the Company, a position from which it cannot be outvoted. A casting vote would allow a Party to win alone; this arrangement ensures the Party funding the Company cannot lose.
- 11.3 A quorum shall be three directors, including at least one director appointed by each of two different Parties.

- 11.4 Where Medbury and Dr Kpaduwa take different positions on a tier two matter, the matter shall be referred to them to resolve before any vote is taken, and CFA shall facilitate that discussion rather than cast a deciding vote. A vote shall be taken only once they have had the opportunity to resolve it, or after fourteen days.
- 11.5 The Board shall meet quarterly, and a monthly operating review shall be held with the executive and minuted. Monthly management accounts, a clinical quality and complications report and progress against the current Stage shall be circulated to all Parties five business days before each review, in the same form and at the same time.
- 11.6 A fifth, independent director may be appointed by unanimous agreement of the shareholders.

12. ROLES AND RESPONSIBILITIES

- 12.1 The Company performs its own functions through its own officers and employees. A function is performed for the Company by a Party only where this Memorandum or a written agreement approved by the Board so provides.
- 12.2 Schedule 1 records, for each function, whether it is performed by the Company itself or by a Party for the Company, and identifies the person accountable to the Board. Accountability for a function shall not be shared.
- 12.3 CFA's holding is not granted for having been appointed operator. It divides in two. 40 per cent of it is founding equity, vesting on the Board's acceptance of each founding deliverable at Schedule 5, each of which has a date. The remaining 60 per cent is operating equity, and vests only as CFA actually runs the Company, in equal instalments against the measures at Schedule 4A, measured annually by the Board. Those measures are written now and not deferred, for the same reason the Clinical Director's are: no party should be asked to accept conditions that have yet to be drafted.
- 12.4 If CFA ceases to operate the Company, or fails against those measures, the unvested operating equity does not vest and is not paid for. CFA does not hold equity for a job it is no longer doing, and the Parties record that this is CFA's own proposal.
- 12.5 The Company shall appoint its own day-to-day lead, recruited by CFA and appointed by the Board under clause 10.3(c). The arrangements that apply until the Company is fully staffed are set out in Part C of Schedule 1.

13. RELATIONSHIP WITH THE MEDBURY GROUP

- 13.1 **Exclusivity is confined to surgery.** The Company shall be the exclusive provider of cosmetic and plastic surgery to the Group in Nigeria, and the Group shall not establish, acquire or partner into a competing cosmetic or plastic surgery service while this clause subsists. Aesthetics is deliberately excluded from that exclusivity, the field being too broad to close off to a diversified healthcare group and already practised across a number of the Group's settings.
- 13.2 For non-surgical aesthetics the Group instead gives the Company a right of first refusal. Where a Group business wishes to add or expand a non-surgical aesthetics service it shall first offer the Company the opportunity to provide it on terms no less favourable, and may proceed independently only if the Company declines or cannot meet the requirement within an agreed period.
- 13.3 **The restriction is reciprocal.** For so long as it remains a shareholder each Party is bound in the same way: it shall not establish, acquire, clinically lead or partner into a competing cosmetic or plastic surgery service in Nigeria, and shall channel such activity through the Company. Nothing in this clause touches the Los Angeles practice of Dr Kpaduwa or anything she does outside Nigeria,

the Group's existing wellness, occupational health, diagnostic and pharmaceutical lines, or CFA's advisory work for third parties save that CFA shall not structure or operate a competing cosmetic or plastic surgery clinic in Lagos.

- 13.4 That exclusivity is conditional on performance and is not given indefinitely. It shall fall away, at Medbury's election and on notice, if the Company fails to commence trading by a longstop date agreed in the Shareholders' Agreement; ceases to hold any registration or licence required under clause 15.11; is unable to accept referrals from the Group within a service level agreed in the referral protocol, on more than an agreed number of occasions; falls below the clinical or service standards agreed at the review under clause 14.2; or is in material breach of this clause. A diversified healthcare group should not be shut out of a field of practice across a whole country by a venture that is not serving it.
- 13.5 Where exclusivity falls away, the Company continues and the other provisions of this Memorandum are unaffected. Medbury regains the freedom to meet demand the Company cannot meet, and shall not use that freedom to compete for the Company's existing patients.
- 13.6 Referrals shall be on arm's length terms recorded in a written referral protocol at agreed rates, so that the accounts of each are true and no clinical decision is influenced by an internal transfer price.
- 13.7 Any other service provided by the Group to the Company, including any transitional shared back office service, shall be provided under a written agreement at agreed rates, shall be approved as a related-party contract under clause 10.6(f) with Medbury not voting, and shall be terminable by the Company on reasonable notice. No such service shall be assumed, and none shall continue after the Company has appointed its own staff to the function unless the Board resolves otherwise.
- 13.8 The independence of the Company from the Group is dealt with in clauses 3.6 and 3.7, and what the Company pays for premises and equipment in clause 9.14.

14. STAGES AND REVIEW

- 14.1 The Company shall be developed in stages:

Stage	Period	Object
Stage 1	From the date of this Memorandum to the Review Date	Proof of the product, traded from the Stage 1 facility under clause 3.4. Incorporation; the hiring and training of the core team; the conclusion of the partner theatre arrangement; the definition of the Stage 1 treatment menu and its delivery by clinicians registered in Nigeria; the completion of the first documented cases; and the establishment of the enquiry pipeline. The tasks are at Schedule 2
Stage 2	2027	The opening of the base at The Lyfe Place; the widening of the clinical team; the growth of the proportion of the menu delivered to standard by the team; and the attainment of sustainable trading
Stage 3	Thereafter	Growth, including additional capacity, a further site or a further city, proposed by the Clinical Director and the executive and requiring unanimous agreement under clause 10.4(g)

- 14.2 On the Review Date the Parties shall review the Company against clinical outcomes and complications; patient experience; the standard held, as certified by the Clinical Director; the proportion of the menu delivered to standard by the trained team; revenue, contribution and cash against the approved plan; and the enquiry pipeline and its conversion.

- 14.3 Following that review the Parties may agree to proceed to Stage 2, to extend Stage 1, or to discontinue the venture. If the Parties agree to discontinue they shall do so cooperatively, with continuity of patient care as the first consideration, and no Party shall be committed beyond the point of proof.
- 14.4 The Parties agree that the object of the Company is that its standard be institutional. Accordingly each protocol shall be written, version controlled and signed off by the Clinical Director; each clinician shall be trained and credentialed against a documented milestone standard; no treatment shall be offered which the Clinical Director has not approved and none delegated which she has not certified as safe to delegate; a deputy clinical lead shall be identified and in place by the end of Stage 2; and the growth of the team and of the menu shall be proposed by the Clinical Director at the pace at which she judges the standard can be held.

15. **STANDARDS OF CONDUCT**

- 15.1 The Company shall deliver to a written clinical quality standard approved by the Clinical Director and documented in the Company's protocols, which shall be no lower than the standard she applies in her own practice and shall in all cases comply with Nigerian law and applicable professional standards. Where local market practice falls below that documented standard, the documented standard prevails. For the avoidance of doubt, the standard is the one documented in the protocols and not every feature of any other practice.
- 15.2 No clinical image or patient account shall be published without the specific, written and revocable consent of the patient, recorded and auditable. Consent to treatment and consent to publication are separate, and neither shall be a condition of the other.
- 15.3 Results published by the Company shall be the Company's own cases. The building of a documented body of results in the patient population the Company treats is a stated object of its content.
- 15.4 No treatment, discount or service shall be exchanged for promotion, coverage or endorsement, and the Company shall not enter into influencer partnerships.
- 15.5 The Clinical Director shall hold registration with the Medical and Dental Council of Nigeria and a current practising licence before she treats any patient of the Company. She is progressing that registration, and CFA shall support it and report its completion to the Board.
- 15.6 Until it is in place she shall not treat any patient in Nigeria, shall not be held out as entitled to do so, and shall not supervise clinical work performed in Nigeria. Her contribution in that period is the authorship of protocols, the design of the training programme, the setting of the clinical standard and the governance of quality, none of which requires registration, and clinical delivery is by clinicians registered in Nigeria working to her protocols.
- 15.7 The Company shall not hold the Clinical Director out as entitled to practise otherwise than as her registration permits, and shall not offer any treatment she cannot lawfully deliver at the site at which it is offered. The Company shall in addition appoint a Nigerian-registered practitioner as its medical director of record for each site it operates, that person meeting the requirements of the Council and of the Health Facility Monitoring and Accreditation Agency. That appointment is distinct from the role of Clinical Director, and a medical director's certificate may be used for one facility only at any time.
- 15.8 **Clinical governance is not the practice of medicine on a patient.** The Nigerian-registered clinician who treats a patient, and the medical director of record, remain independently and

- personally responsible for diagnosis, treatment, informed consent, execution, follow-up and compliance with their own professional obligations. The Clinical Director's authorship of a protocol, credentialing of a clinician, approval of a treatment for the menu or setting of the standard does not constitute supervision of, or participation in, any individual patient encounter, and shall not be construed as such, unless she expressly undertakes that encounter herself.
- 15.9 The Company shall indemnify the Clinical Director against claims arising from the acts or omissions of others in the course of the Business, save for her own fraud or wilful misconduct, and shall advance her defence costs. It shall maintain, at its own cost and for so long as any claim may be brought: medical indemnity expressly covering her activities as Clinical Director as distinct from her activities as a treating clinician; directors' and officers' cover; and run-off cover for a period agreed in the Shareholders' Agreement after she ceases to hold the role. Evidence of cover shall be produced to her annually.
- 15.10 The Company shall use only products registered by the National Agency for Food and Drug Administration and Control, no product being lawfully importable, sold or used in Nigeria otherwise than as registered under the NAFDAC Act Cap N1, Laws of the Federation of Nigeria 2004. Where a product used in the Clinical Director's United States practice is not registered in Nigeria, she shall either approve a registered equivalent or the treatment shall not be offered, and clause 15.1 shall be read accordingly.
- 15.11 Before treating any patient the Company shall hold incorporation and tax registration; health facility registration with HEFAMAA for each site; full registration of each clinician with the Medical and Dental Council of Nigeria or the Nursing and Midwifery Council of Nigeria as applicable, verified before practice; the Nigerian practising registration and current practising licence of the Clinical Director on whichever route clause 15.6 establishes, together with imported product; prescription-only medicines held under a named resident prescriber; a written complications protocol, with hyaluronidase and an anaphylaxis kit stocked and in date at each site at which injectable treatments are performed; compliance with the Nigeria Data Protection Act 2023; and medical indemnity for the Company and each clinician together with public liability cover at each site.
- 15.12 The position of the American Society of Plastic Surgeons on itinerant surgery, issued 19 September 2024, governs the surgical practice of the Company. It makes the operating surgeon responsible for the continuity of the patient's care including the treatment of complications. Accordingly the Company shall appoint a plastic surgeon resident in Lagos, board certified or equivalently qualified, to whom care may be transferred on the patient's informed consent, and no surgical procedure shall be offered before that appointment is made.
- 15.13 The Company shall contract with one or more licensed partner facilities for the use of a theatre, anaesthesia, recovery and any overnight or escalation capacity its cases require. Each such arrangement shall be contracted by the Company and not by any Party individually, and no Party shall negotiate with or commit to a facility on the Company's behalf without a written brief and a mandate from the Board. Operating privileges may be held personally by a clinician where the facility so requires, but the patient, the record, the consent and the revenue belong to the Company. Each arrangement shall settle theatre access and scheduling, anaesthesia and recovery, escalation and admission, fees and the flow of funds, and the terms on which each party's name may be used. Such arrangements are non-exclusive to the Company unless the Board agrees otherwise.
- 15.14 Where a commercial opportunity conflicts with the clinical or ethical standard, the standard shall prevail and the decision of the Clinical Director shall be final.

16. CONFIDENTIALITY

16.1 Each Party shall keep confidential this Memorandum, its terms, and all information disclosed to it by another Party in connection with the venture, and shall not disclose the same otherwise than to its professional advisers or as required by law.

16.2 This clause continues in force after a Party ceases to participate in the venture.

17. STATUS OF THIS MEMORANDUM

17.1 Clause 5 (intellectual property and data), clause 9.21 (costs already incurred), clause 16 (confidentiality), this clause 17, clause 18 (governing law and disputes) and clause 19 (general) are intended to be legally binding on the Parties from the date of this Memorandum.

17.2 The remaining provisions record the Parties' intentions and their agreement in principle. They are not intended to create legal relations and shall take effect only on execution of the Shareholders' Agreement and the constitution of the Company.

17.3 Nothing in this Memorandum obliges any Party to enter into the Shareholders' Agreement or to incorporate the Company.

17.4 Each Party shall bear its own costs of negotiating and preparing this Memorandum and the Shareholders' Agreement. The costs of incorporation shall be borne by the Company.

18. GOVERNING LAW AND DISPUTES

18.1 This Memorandum, and any dispute arising out of or in connection with it, are governed by the laws of the Federal Republic of Nigeria.

18.2 The Parties shall seek to resolve any dispute by negotiation between the principals within fourteen days, failing which by mediation before a jointly appointed mediator, and failing which by arbitration in Lagos under the Arbitration and Mediation Act 2023.

19. GENERAL

19.1 This Memorandum may be executed in counterparts and by electronic signature, each of which is an original and all of which together constitute one instrument.

19.2 This Memorandum supersedes all prior drafts, term sheets and discussions between the Parties concerning the venture, and may be varied only in writing signed by all Parties.

19.3 No Party may assign or transfer the benefit of this Memorandum without the written consent of the others.

19.4 Nothing in this Memorandum creates a partnership between the Parties or constitutes any Party the agent of another.

SCHEDULE 1

Functions and accountability

This Schedule is given under clause 12. Part A lists the functions the Company performs itself, Part B those a Party performs for it, and Part C the transitional arrangements until the Company is fully staffed.

Part A: functions the Company performs itself

Performed by the Company's own officers and employees, and accountable to the Board through the Company's day-to-day lead appointed under clause 12.3.

Clinical delivery by the Company's employed clinicians, within their credentialing
Patient reception, coordination, scheduling and aftercare follow-up
Consent taking and record keeping, including consent to publication under clause 15.2
Finance: bookkeeping, management accounts, banking and treasury
Payroll, employment administration and pension and statutory deductions
Statutory filings and corporate secretarial compliance
Insurance placement and renewal, including medical indemnity and public liability
Site level stock control, cold chain and expiry management
Patient feedback and complaints
Health and safety, clinical waste and incident reporting
Data protection compliance under the Nigeria Data Protection Act 2023, and the response to data subject requests

Part B: functions a Party performs for the Company

Each is performed under the written agreement identified against it, and is accountable to the Board.

Function	Performed by	Under
Clinical standard, protocols and treatment algorithms	Dr Kpaduwa	Clause 10.2 and the licence under clause 5.2
Credentialing of clinicians and sign-off before independent practice	Dr Kpaduwa	Clause 10.2
Training of the clinical team	Dr Kpaduwa	Clause 14.4; scheduled and organised by CFA
Delivery of the advanced and surgical work	Dr Kpaduwa	Clinical engagement terms under clause 9.2
Brand direction, messaging, and approval of clinical content	Dr Kpaduwa	Clause 10.2 and the licence under clause 5.2
Formation, licensing and regulatory establishment	CFA	Services agreement, clause 9.15
Recruitment of the Company's team	CFA	Services agreement, clause 9.15
Implementation of the operating systems and the patient record	CFA	Services agreement, clause 9.15

Function	Performed by	Under
Marketing operations, funnel and content production	CFA	Services agreement, clause 9.15; content approved by Dr Kpaduwa
Procurement, importation and supply chain	CFA	Services agreement, clause 9.15; specification set by the Clinical Director
Programme management and reporting to the Board	CFA	Services agreement, clause 9.15
The base and the fit-out works, and the equipment	Medbury	As owner of the premises, under clause 9.14
Introductions to host institutions, referring hospitals and corporate accounts	Medbury	Clause 6.2. The Company contracts with the institution under clause 15.13
Referral of aesthetics and cosmetic surgery work from the Group	Medbury	Referral protocol under clause 13.2

Part C: transitional arrangements

Until the Company's day-to-day lead is appointed, and until the Company has appointed its own finance and administrative staff, CFA shall carry the Part A finance, payroll, filings and insurance functions under the services agreement referred to in clause 9.15.
Medbury may provide any Part A function as a transitional shared service, but only on the terms and subject to the approvals in clause 13.7.
Where a person employed within the Group is to work for the Company, that person shall either be employed by the Company or seconded to it on written terms approved by the Board under clause 3.7. No person shall perform a Company function without one or the other being in place.

SCHEDULE 2

Immediate work plan

Periods run from the date of this Memorandum unless otherwise stated.

Action	Owner	By when
Shortlist, search and agreement of the House Brand	CFA prepares; all Parties agree	10 business days
Incorporation, bank accounts and statutory registrations	CFA	10 business days from selection of the name
Instruction of lawyers on the Shareholders' Agreement and constitution	CFA instructs; all Parties review	2 weeks
Execution of the licence under clause 5.2	KP Plastic Surgery, procured by Dr Kpaduwa, and the Company	With the Shareholders' Agreement
Written enquiry to the Medical and Dental Council and opinion on the registration route under clause 15.5	CFA	Enquiry within 7 days; report within 28 days
Definition of the Stage 1 treatment menu	Clinical Director	7 days
Establishment budget and twelve month operating budget	CFA prepares; Board approves	2 weeks
Recruitment of two surgical nurses and one care coordinator	CFA, with Medbury sourcing internally	Offers issued within 3 weeks
Training programme: discovery call, patient management, the patient journey and protocols	Clinical Director; organised by CFA	On appointment of the team
Review of United States enquiry and audience data, and compilation of the Nigerian interest list	Dr Kpaduwa's marketing team with CFA	At the marketing meeting
Written brief for the partner theatre arrangement, and thereafter the meeting with Euracare	CFA drafts; all Parties attend	Brief first; meeting within 2 weeks
Three to five index procedures, consented, documented and photographed to standard, performed by a clinician registered to perform them	Coordinated by CFA to the Clinical Director's protocols	Before the Review Date, subject to clause 15.6
Marketing funnel: channels, referral protocol and content calendar	CFA; approved by Dr Kpaduwa	3 weeks
The Lyfe Place base: brief, drawings and fit-out programme	Medbury; project managed by CFA	4 weeks
Occupancy terms for The Lyfe Place	Medbury and the Company	With the Shareholders' Agreement
Regulatory: facility registration, the Clinical Director's licence and product compliance	CFA	Commenced within 2 weeks
The review under clause 14.2	All Parties	November 2026

SCHEDULE 3

Matters to be settled in the Shareholders' Agreement

The following are not settled by this Memorandum. None of them prevents the work at Schedule 2 from proceeding.

	Matter	To be settled
1	The shareholding	Confirmation of the percentages at clause 7.1
2	The holding vehicle	Whether Dr Kpaduwa holds her shares personally or through a Nigerian company she owns, both of which preserve the position in clause 7.9
3	The registration route	The Clinical Director's route to registration on the Council's answer under clause 15.5, and what it means for the Stage 1 timetable and for who performs the first cases
4	Cash subscription	Whether any Party subscribes cash beyond its contribution under clause 6, and the consequent adjustment
5	Build-up on holdings	Whether any holding is subject to a build-up or milestone arrangement, and on what basis
6	Clinical remuneration	The terms on which the Clinical Director and the clinical team are remunerated under clause 9.2
7	Funding terms	The establishment budget, the drawdown schedule and the priority return under clause 9.13
8	Occupancy and equipment	The rate and the term under clause 9.14
9	CFA's services agreement	Scope and terms under clause 9.15
10	The operating calendar	The Stage 1 and Stage 2 calendars against which the Company plans, books and staffs
11	Interim delivery	Where non-surgical treatment is delivered before the base opens
12	The partner theatre	Which facility, and the terms on which the Company's cases are performed there under clause 15.13
13	Continuing commitments	Whether any commitment binds a Party after it ceases to be a shareholder, and for how long
14	Transfers and leaver terms	Pre-emption, tag and drag rights, valuation, and the treatment of a holding on departure
15	Deadlock	The mechanism for resolving a fundamental deadlock

SCHEDULE 4

Derivation of the shareholding

The method, agreed between the Parties, is to cost the set-up, value what each Party brings at what a third party would charge for it, deduct whatever cash that Party is actually paid for it, and let equity follow the residual. A Party paid the full market rate for a thing has contributed nothing to the balance sheet in respect of it. A Party that forgoes cash has subscribed with the money it did not take.

All figures NGN millions, over the first three years, on cumulative revenue of NGN 1,200m. Every figure is a benchmark to be replaced with the Parties' own numbers, at which point the percentages recompute.

Medbury Healthcare Group

Contribution	Valued on	At market	Cash paid	Net
Working capital funded to trading	Cash advanced as needed, at risk and unrecoverable if the venture fails	72	nil	72
Equipment capital expenditure	Cost of the equipment the service runs on	54	nil	54
Access to the partner theatre, negotiated and held by Medbury	2.5 per cent of the NGN 480m of surgical revenue it enables, below the 5 per cent a brand licence carries because the Company still pays market per-case theatre fees	12	nil	12
Rent forgone on the Medbury premises the Company occupies	NGN 72m a year of guaranteed third-party income across the Stage 1 facility and thereafter the base, 45 per cent of it waived over the window	216	119	97
Net contribution		354	119	236

Dr Chinwe Kpaduwa

Contribution	Valued on	At market	Cash paid	Net
Licence of the name and the demand it creates, granted at launch	5 per cent of the NGN 400m of revenue the name attracts, not of the whole book	20	nil	20
The clinical standard and protocols	Cost of developing an equivalent standard from nothing. Delivered at launch, so it is issued at incorporation and is not contingent	25	nil	25
Training and credentialing of the team	Market cost of an equivalent programme over the window	24	nil	24
Surgeon's fees forgone	Market fee of NGN 168m, of which she draws 50 per cent in cash	168	84	84
Net contribution		237	84	153

Consult for Africa

Contribution	Valued on	At market	Cash paid	Net
Structuring, incorporation and the agreements	Market advisory cost	15	nil	15

Contribution	Valued on	At market	Cash paid	Net
Regulatory establishment	Market cost of the licensing pathway	0	nil	0
Recruitment of the founding team	Market search fees	11	nil	11
Configuration and implementation of the operating systems	Cost of the build the Company keeps: configuration, data migration and training. The platform licence itself is NOT here, being paid for within the management fee and therefore compensated	18	nil	18
Management fee forgone	Fee of 10 per cent of revenue against a market range of 8 to 20 per cent, charged in full at 10 per cent, so nothing is forgone and no equity arises	120	120	0
Net contribution		164	120	44

Party	Net contribution	Share	Held
Medbury Healthcare Group	236	54.5%	60%
Dr Chinwe Kpaduwa	153	35.4%	30%
Consult for Africa	44	10.2%	10%
Total	433	100.0%	100%

Two points on the valuation of the name, which is the line most open to challenge. A royalty of 5 per cent is the market rate for an established brand in its own market. In Nigeria the marks have no trading history and no local recognition, so the royalty is applied to the revenue the name can be shown to attract, being NGN 400m of the NGN 1,200m, and not to the whole book. That values the licence at NGN 20m rather than NGN 60m. The identical basis is applied to Medbury's referral flow, so that the capability each Party claims is measured the same way. The name is valued once. It is not counted again as demand generated, those being two ways of pricing the same pull.

SCHEDULE 4A

Vesting measures

Given under clause 7.16. These are the measures themselves rather than an undertaking to agree measures later. No measure may be added, removed or altered without the written consent of the Party it applies to.

Dr Chinwe Kpaduwa

Measure	What is required
On incorporation	The licence of the Marks and the clinical standard and protocols, delivered at launch. Issued in full and not contingent
Protocols	Each protocol on the Stage 1 menu written, version controlled and signed off
Registration	Registration with the Council and a current practising licence in place
Presence	Three visits a year of not less than ten working days each, excluding two months notified in advance, with at least two operating lists per visit once registration permits
Governance	Four remote clinical governance sessions a month and quarterly protocol review
Training	Each clinician trained and signed off against the documented milestone standard, being ten observed and ten supervised cases per modality before independent practice
Succession	A deputy clinical lead identified and in place by the end of Stage 2

Consult for Africa, operating tranche

Measured annually by the Board. The founding tranche vests on acceptance of the deliverables at Schedule 5.

Measure	What is required
Formation	Company incorporated, bank accounts open, statutory registrations complete
Regulatory	Every item at the compliance clause held, so the Company may lawfully treat
Team	The six founding appointments made and in post
Systems	Patient record, booking, clinical documentation and reporting live and in use
Reporting	Monthly management accounts to the Board within ten working days of month end, and the clinical quality and complications report with them
Budget	Operating expenditure within the approved budget, or variance explained and accepted by the Board
Handover	The Company's own day-to-day lead appointed, and the Part C transitional functions handed over on the agreed schedule

Medbury Healthcare Group

Equity referable to capital issues as the capital is actually advanced against the approved budget. Equity referable to rent forgone vests as it is forgone, month by month, over the window at clause 7.6.

SCHEDULE 5

What CFA's contributions cover, and over what period

Given in answer to the question of what each line at Schedule 4 buys. Figures are budget estimates to be confirmed against quotations before the Shareholders' Agreement is executed, and any underspend reduces the contribution and the holding with it.

Regulatory establishment, NGN 0m

Establishment only. It runs from signature until the Company holds everything at clause 15.10 and may lawfully treat a patient. Annual renewals after that are an operating cost of the Company and not a CFA contribution.

Item	What it covers	NGN 000
Incorporation at the Corporate Affairs Commission	filing, stamp duty and agent	150
Tax registration	TIN is free; agent time only	50
Health facility registration with HEFAMAA	cosmetic procedure centre, VERIFIED at the published rate	50
Clinical team registration	MDCN and NMCN for the founding clinicians	250
The Clinical Director's registration	Council application and licence fees. She is progressing this herself	150
NAFDAC	NIL where the Company buys from licensed Nigerian distributors already holding registration for those products, the distributor being the local agent. Registering independently would cost materially more and there is no reason to	0
Data protection	data controller registration with the Commission	100
Trade marks	House Brand in three classes, plus recording the registered user	400
Professional and filing costs	counsel and agents across the pathway	300
Total		1,450

Recruitment of the founding team, NGN 11m

Six appointments, being the team the Company needs to open and to satisfy clause 15.11. Priced at market search fees for the roles. It covers the founding team only; hiring after the Company is open is an operating cost.

Role	Number
Two surgical nurses	2
Care coordinator	1
Resident aesthetic physician	1
Resident plastic surgeon, clause 15.11	1
The Company's day-to-day lead, clause 12.3	1

Role	Number
Total appointments	6

Structuring, NGN 15m, and systems, NGN 18m

Structuring covers this Memorandum, the Shareholders' Agreement, the constitution, the licence at clause 5.2 and the services agreement, to execution. Systems covers the configuration of the patient record, booking, clinical documentation and reporting, the data migration and the training of staff on them, to go-live. It is the build the Company keeps. The licence of the underlying platforms earns CFA nothing, per clause 5.5.

What CFA actually contributes, and what it recognises

The figures above are deliverable prices and pass-through costs. They price no professional time at all. Because the fee at clause 9.16 is charged on revenue and there is no revenue before launch, every hour CFA works between signature and opening is uncompensated. Priced on CFA's ordinary basis of resourced days at published rates plus a programme rate, that time is set out below.

Workstream	Days	NGN m
Structuring, negotiation and the agreements	45	29.2
Regulatory establishment, running the pathway	38	11.0
Recruitment of the six founding roles	42	11.8
Systems configuration, migration and training	63	12.0
Commercial and financial modelling	24	13.2
Programme mobilisation to launch	36	13.3
Professional time, 248 days, with the programme rate		100

Two further contributions were carried at nil in earlier drafts and are recorded here.

Contribution	NGN m	Basis
Subordinated risk on the deferred fee	43	CFA defers NGN 72m ranking behind Medbury's capital and priority return, paid only if the venture succeeds. Risk borne is the difference between face and present value
Origination: CFA brought the parties together and built the venture	18	
Access to a diaspora clinician network, which is where the resident plastic surgeon required by clause 15.11 realistically comes from	15	
Regulatory pathway knowledge, demonstrated in this Memorandum rather than asserted	8	

Taken together CFA's contribution is about NGN 212 m. **CFA recognises NGN 44 m of it, being some 21 per cent, against the 30 per cent at which Medbury's and Dr Kpaduwa's valuations are recognised.** On

the framework CFA would hold about 16 per cent. It holds 10. The difference is not an oversight and it is not charged for: it is recorded so that the Parties know what the operator is contributing and choosing not to count.

The regulatory line is the one most often mistaken for a fee. It is not. Every naira of it goes to the Corporate Affairs Commission, HEFAMAA, the Councils, NAFDAC, the Trade Marks Registry or an agent, as itemised above. CFA's own time running that pathway sits in the table above and is not charged.

What the management fee at clause 9.16 pays for

The fee shall be decomposed in the services agreement and reported to the Board each quarter in three parts, so that no part of it is mistaken for another: the cost of personnel CFA provides who work wholly for the Company, which is reimbursement and not margin; the cost of the platforms and third-party services CFA supplies; and CFA's management margin, being the residue. Only the third is CFA's remuneration, and only the third is to be compared with the market rate for management services. CFA takes no equity for personnel whose cost the Company reimburses, and its holding at clause 7.1 is for the founding work at Schedule 4 alone.

The fee is not a profit share and it is not a second charge for the founding work above. It funds the operating team and the systems that run the Company day to day: the executive and operations staff CFA provides until the Company employs its own under Part C of Schedule 1, the use of CFA's platforms, marketing operations, procurement and supply chain including foreign exchange management, management reporting to the Board, and CFA's supervision of the whole. It steps down as the Company's own staff take those functions on, and an itemised statement of what it covers is annexed to the services agreement.

SCHEDULE 6

What the Company needs, and what the ceiling is for

The ceiling at clause 9.3 is not a target. It is an outside limit, and the Company draws only what the approved budget requires. This Schedule sets out the requirement built up from the work rather than assumed, so that the ceiling can be tested against it.

Stage	Requirement	NGN m
Stage 1	Regulatory fees paid direct to the regulators, Schedule 5	1.5
Stage 1	Core team for three and a half months	5.3
Stage 1	Opening consumables and injectables stock	4.0
Stage 1	Equipment gap at the existing clinic, to be surveyed	6.0
Stage 1	Launch marketing and content production	6.0
Stage 1	Operating float	3.0
Clinic	Two consulting rooms and examination couches	2.5
Clinic	Standardised clinical photography	1.5
Clinic	Minor procedure room, lights, trolley and autoclave	6.0
Clinic	Emergency: crash trolley, defibrillator, oxygen, hyaluronidase and anaphylaxis stock	3.0
Clinic	Information technology hardware	2.0
Clinic	Power backup	5.0
Surgical kit	Power-assisted liposuction and fat harvesting	12.0
Surgical kit	Closed fat processing and grafting system	6.0
Surgical kit	Intraoperative ultrasound, mandatory for gluteal fat grafting	8.0
Surgical kit	Plastic surgery instrument sets: face, breast, body	6.0
Surgical kit	Electrosurgery unit	2.0
Working capital	First year trading loss on the revenue ramp	30.0
Working capital	Receivable and stock timing	10.0
Working capital	Contingency	6.0
Total		125.8

The fit-out of the base is Medbury's investment in its own estate, recovered through the occupancy terms at clause 9.14, and is not a cash requirement of the Company.

The device roadmap, which is deferred and not in the requirement above

None of the following is in the cash requirement. Each is revenue-generating and is added from trading income or leased once the diary justifies it. The ordering is clinical rather than aspirational: most patients of this Company will be Fitzpatrick IV to VI, and several of the best known platforms are the wrong

instruments for that skin. Selecting by brand recognition rather than by skin type is a clinical risk before it is a commercial one, and the Clinical Director's approval under clause 10.2 governs.

Platform	NGN m	Position	Why
Long-pulsed Nd:YAG 1064nm	35	BUY FIRST	The gold standard for hair removal and vascular work on Fitzpatrick V to VI. Penetrates past the melanin-rich epidermis, so it treats rather than burns
Radiofrequency microneedling, Morpheus8 class	30	BUY	Radiofrequency is colour-blind: it does not target melanin, so it is among the safest resurfacing and tightening options on darker skin
Micro-focused ultrasound lifting, Ultherapy or Sofwave class	40	BUY	Delivers energy below the epidermis, so skin tone is not a limiting factor
Picosecond Nd:YAG, PicoWay or PicoSure class	45	BUY LATER	For pigment. Picosecond pulses work photomechanically rather than thermally, which reduces heat spread into surrounding melanin
Hydradermabrasion, HydraFacial class	12	BUY LATER	No light, safe across every skin type, and a strong volume and retention driver
Electromagnetic body contouring, Emsculpt Neo class	45	OPTIONAL	Not light-based, so skin type is irrelevant. A non-surgical body line that complements rather than competes with the surgical offer
Intense pulsed light, including Sciton BBL and M22 class	deferred	DO NOT BUY	Broad-spectrum light is too aggressive on melanin-rich skin. Reported hyperpigmentation in about 60 per cent and hypopigmentation in about 20 per cent of darker-skinned patients. Wrong instrument for this population
Alexandrite 755nm	deferred	DO NOT BUY	Strongly melanin-absorbing and not appropriate above Fitzpatrick IV
Ablative fractional CO2	deferred	NOT YET	Can be used on darker skin by experienced hands at adjusted settings, but carries a real post-inflammatory hyperpigmentation risk. Revisit once the resident team is established, not at launch

How the requirement can be reduced

The Board shall consider each of the following before drawing against the ceiling. They reduce the peak funding rather than the total cost of the assets, and they improve the Company's cash profile in the period when it is least able to carry one.

Measure	NGN m	Effect
Lease the liposuction and ultrasound rather than buy	20	converts capital expenditure to a monthly operating cost
Refurbished units, warranted, for two items	6	ordinary practice in the sector
Peak requirement if all three are taken	100	

EXECUTION

The Parties have signed this Memorandum on the date first written above.

SIGNED for and on behalf of
MEDBURY HEALTHCARE GROUP

Dr Itunu Akinware, Group Chief Executive

Date

SIGNED for and on behalf of
DR CHINWE KPADUWA

Dr Chinwe Kpaduwa, in her personal capacity

Date

SIGNED for and on behalf of
CONSULT FOR AFRICA

Dr Adebawale Odulana, Founding Partner

Date

JOINDER, limited to clause 5.2

KP Plastic Surgery joins this Memorandum for the sole purpose of the licence of the Marks at clause 5.2 and for no other purpose. It is not a shareholder in the Company, assumes no obligation under any other clause, and its execution here does not make it a Party.

SIGNED for and on behalf of
KP PLASTIC SURGERY

for and on behalf of KP Plastic Surgery

Date
